BASELINE → TREATMENT → FOLLOW-UP

Required and conditional checks across your treatment

Required checks are shown explicitly. electrocardiogram (ECG), additional urine tests and anticoagulant tests apply only when indicated. Scans and tumour markers follow your individual cancer plan; this page does not prescribe a universal surveillance interval. Confirm who books and reviews each test.

Check	Before starting	During treatment	Between visits	Review / change	After treatment
Blood counts / kidneys / liver	Required baseline blood count, kidney and liver tests	Required before each cycle; results reviewed before treatment	Earlier tests if unwell, dehydrated or toxicity develops	Recovery and results guide delay or dose adjustment	Follow unresolved abnormalities as agreed
dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) / fluoropyrimidine safety	DPD test before fluorouracil or capecitabine unless already tested	The result guides the starting plan; it does not exclude all severe toxicity	Unexpected severe early mouth sores, diarrhoea or illness needs urgent assessment	The team reviews suspected severe reactions before further treatment	Keep the result and any toxicity history in your records
Blood pressure / urine / healing	Required baseline blood pressure (BP), urine protein and cardiac history/examination	BP before each cycle and after treatment; urine protein each cycle	Report severe headache, bleeding, swelling, reduced urine or poor healing	Abnormal urine/BP results may need further tests or a treatment hold	Coordinate surgery, dental procedures and wound healing with oncology
Heart assessment	Required cardiac history/ examination; ECG if clinically indicated. Consider an echocardiogram after prior anthracycline chemotherapy or chest-wall radiotherapy.	Report any new chest pain or breathing change	Chest pain, collapse or severe breathing difficulty needs emergency assessment	Additional heart tests only when clinically indicated	Follow any identified heart problem; no fixed routine echo series
international normalised ratio (blood-clotting test) (INR) only if taking warfarin	Tell the team about warfarin and arrange a baseline INR	INR weekly until the warfarin dose is stable, then before each cycle	Report bleeding or unusual bruising	Only change anticoagulants as instructed	Continue the individually agreed anticoagulation follow-up

Check	Before starting	During treatment	Between visits	Review / change	After treatment
Response / scans / tumour markers	Agree a baseline disease assessment and treatment aim	Review symptoms, function and clinical benefit	New symptoms may require earlier review	Scans and tumour markers follow the consultant-directed cancer plan	Agree next treatment or follow-up; there is no universal scan interval in this regimen

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00791 v2 · official regimen

Tests in your plan

Carcinoembryonic antigen (CEA) — colorectal plans

If part of your plan

WHEN

For a colorectal cancer plan, if your team uses CEA; your team sets the interval.

A marker trend interpreted alongside symptoms, examination and scans.

Record the baseline, latest result and next date. A marker change alone does not establish progression.

THE WHOLE TREATMENT JOURNEY

Bevacizumab + fluorouracil / folinic acid: treatment and checks

Metastatic colorectal cancer

Day 1: bevacizumab and folinic acid, then fluorouracil bolus and a 46-hour fluorouracil pump. Repeat every 14 days while treatment is helping and remains tolerable. This regimen does not include oxaliplatin or irinotecan.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits	Review / change	After treatment
Blood counts / kidneys / liver NCCP 00791	Required baseline blood count, kidney and liver tests	Required before each cycle; results reviewed before treatment	Earlier tests if unwell, dehydrated or toxicity develops	Recovery and results guide delay or dose adjustment	Follow unresolved abnormalities as agreed

Care / check	Before starting	During treatment	Between visits	Review / change	After treatment
DPD / fluoropyrimidine safety NCCP 00791	DPD test before fluorouracil or capecitabine unless already tested	The result guides the starting plan; it does not exclude all severe toxicity	Unexpected severe early mouth sores, diarrhoea or illness needs urgent assessment	The team reviews suspected severe reactions before further treatment	Keep the result and any toxicity history in your records
Blood pressure / urine / healing NCCP 00791	Required baseline BP, urine protein and cardiac history/examination	BP before each cycle and after treatment; urine protein each cycle	Report severe headache, bleeding, swelling, reduced urine or poor healing	Abnormal urine/BP results may need further tests or a treatment hold	Coordinate surgery, dental procedures and wound healing with oncology
Heart assessment NCCP 00791	Required cardiac history/ examination; ECG if clinically indicated. Consider an echocardiogram after prior anthracycline chemotherapy or chest-wall radiotherapy.	Report any new chest pain or breathing change	Chest pain, collapse or severe breathing difficulty needs emergency assessment	Additional heart tests only when clinically indicated	Follow any identified heart problem; no fixed routine echo series
INR only if taking warfarin NCCP 00791	Tell the team about warfarin and arrange a baseline INR	INR weekly until the warfarin dose is stable, then before each cycle	Report bleeding or unusual bruising	Only change anticoagulants as instructed	Continue the individually agreed anticoagulation follow-up
Response / scans / tumour markers NCCP 00791	Agree a baseline disease assessment and treatment aim	Review symptoms, function and clinical benefit	New symptoms may require earlier review	Scans and tumour markers follow the consultant-directed cancer plan	Agree next treatment or follow-up; there is no universal scan interval in this regimen

What each check is for

1

14-day treatment cycle

Day ward → home pump → review

This combination is used to control metastatic colorectal cancer. It may be part of a maintenance plan after other treatment, but that is not the only possible setting. Your team explains why these medicines are being used now and how benefit will be assessed.

Ask / act: Confirm the phase, prescription, tests and supportive medicines before starting.

Sources: NCCP 00791

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-04 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.33